SACTCHECK · REGIMEN-SPECIFIC SACT CONSENT

PROTOTYPE · GOVERNANCE REVIEW REQUIRED

Trastuzumab emtansine (T-DM1) · metastatic

Previously treated HER2-positive unresectable locally advanced or metastatic breast cancer · NCCP 00206 · version 5 · 21-day cycle

Patient name	DOB	Hospital no.	Date

Treatment context
T-DM1 links HER2 targeting to a chemotherapy payload to help control previously treated advanced breast cancer. Prior trastuzumab and a taxane, plus the full NCCP prior-treatment or early-relapse criteria, must be checked.

Schedule
T-DM1 3.6 mg/kg IV every 21 days, adjusted if required, until progression or unacceptable toxicity. This metastatic course has no fixed 14-cycle endpoint; the postoperative early-breast regimen is a different card.

1. TREATMENT DISCUSSION

- ☐ Purpose, expected benefit and alternatives explained
- ☐ Duration and treatment setting discussed
- ☐ Supportive medicines and urgent contact advice discussed
- ☐ Common, serious and lasting agent-specific risks discussed below
- ☐ Correct HER2 medicine and formulation confirmed
- ☐ Heart and other required monitoring explained
- ☐ Pregnancy / contraception / breastfeeding discussed where relevant
- ☐ Companion cancer medicines reviewed using their own information

2. REGIMEN / AGENT-SPECIFIC MATERIAL RISKS

Trastuzumab emtansine (T-DM1)
HER2-DIRECTED ANTIBODY WITH A CHEMOTHERAPY PAYLOAD

- ☐ Low platelets and bleeding
- ☐ Liver injury, rarely serious
- ☐ Numbness or tingling
- ☐ Heart pumping-function reduction
- ☐ Rare serious lung inflammation
- ☐ Fatigue, nausea and infusion reactions

Discuss expected benefit, alternatives, route, duration, cardiac monitoring, pregnancy / contraception, urgent contacts and individual treatment risks. Add any companion drug using its own regimen information.

Individual aim / benefit, alternatives, material risks and questions:

3. CONSENT CONFIRMATION

- ☐ Expected benefit, important risks and reasonable alternatives discussed
- ☐ Patient questions invited and answered; patient-specific material risks / priorities considered
- ☐ Emergency contact / toxicity advice and written information offered where appropriate
- ☐ Patient understands consent is voluntary and may be withdrawn

Patient / person giving consent _____ Date _____

Clinician obtaining consent _____ Date _____

Prototype only. This one-page form is a regimen-specific consent aid and requires formal governance, consultant and oncology-pharmacy review before clinical use. The current NCCP regimen, local/national consent policy, current medicine information, patient-specific material risks and clinical judgement remain authoritative.
Base regimen: NCCP 00206 v5.

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